To: City of Berkeley Leadership

From: Scott Gilman Department of Health, Housing, and Community Services (HHCS)

Date: March 6, 2026

Subject: FY27 & FY28 Budget Balancing Proposal: 12.5% Reduction Scenario

---

Introduction

This memorandum provides key concepts and Director's thoughts on the Department of Health, Housing, and Community Services (HHCS) plan to achieve a 12.5% General Fund reduction for the FY27 and FY28 biennial budget. This memo serves as a high-level summary; more exhaustive descriptions and justifications for each line item and contract are maintained within the detailed budget workbook.

Faced with significant fiscal headwinds in both GF and multiple specialty funds, our department has engaged in a rigorous, principle-driven process to identify $4.4 million in annual GF adjustments and a variety of other actions to correct specialty fund deficits. Due to the severity of the structural deficit, the recommendations are deep. While efforts were made to reduce the impact, the recommendations from HHCS are very painful and will be met with considerable community concern.

Immediate Challenges with Specialty Funding

1. Aging Services Division, Alameda County Area Agency on Aging (AAA) On March 5th, AAA provided notification of an impending cut to the Information & Assistance (I&A) program funding, effective July 1, 2026, totaling $156,803. This program is funded through the federal Older Americans Act funds that pass through to the Alameda County Area Agency on Aging (AAA). Berkeley has been a recipient of these funds for well over two decades. Funds from this program have consistently funded the high-quality social services that Berkeley provides to the older adult community. The loss of these funds is devastating to the services that Berkeley provides. Our fiscal team is actively developing a comprehensive plan to mitigate the effect of this reduction. We are recommending reallocating Fund 320 funds from a vacant position to support maintaining existing staff on this grant.

2. California Strengthening Public Health Initiative (CASPHI) Berkeley Public Health Division also received notification of an impending cut to the program funding, effective February 11, 2026. While the announcement came out in January, this cut has been the subject of a federal lawsuit and temporary restraining order. While we don't have the final notice, we believe based on input from our state association that the funding will be cut.

The program was designed to strengthen the public health workforce using data-driven planning, cross-sector partnerships, and an explicit commitment to health equity. Funding has expanded HHCS's capacity to address disparities by supporting the creation of the Senior Health Management Analyst position, which has been essential for workforce development, data metric development, mentorship, and advancing the department's Equity Plan. Cutting this program would jeopardize critical workforce expansion efforts and delay key deliverables, including the launch of a public-facing, equity-focused data dashboard aligned with the 2025 Community Health Improvement Plan. The estimated annual budgetary impact of this reduction on our division is estimated at $650,928 total ($130,186 annually). Our fiscal team is actively developing a comprehensive plan to mitigate the effect of this reduction. Retaining the Senior Health Management Analyst position is critical to maintain HHCS's data requirements, workforce, and equity capacity that directly supports Berkeley's public health priorities.

3. CalFresh Healthy Living (CFHL) On March 6th, Berkeley Public Health Division received notification of an impending cut to the CalFresh Healthy Living (CFHL) program funding, $162,390 annually effective May 1, 2026. Public Health administers a portion of the United States Department of Agriculture (USDA) funded Supplemental Nutrition Assistance Program, known in California as CFHL. The purpose of the Grant is to provide allowable nutrition education and obesity prevention activities and interventions for low-income Californians. The focus of the project is to upstream public health approaches and individual and organizational health promotion to help the CFHL target audience establish healthy eating habits, a physically active lifestyle, and for primary prevention of disease. The target audience for CFHL is specifically SNAP participants and other low-income individuals who qualify for SNAP (CalFresh Food) benefits or other means-tested Federal assistance programs. The program's core activities in Berkeley include direct education for children aged 0-5, aiming to increase healthy food and water consumption, boost physical activity, and reduce screen time, with an annual reach of approximately 650 children. Concurrently, direct education for adults targets teachers, staff, and parents (ages 18-59), reaching around 300 individuals annually. The program also conducts monthly healthy food demonstrations for older adults (59+), typically reaching 320 individuals annually, and provides indirect education through healthy cooking resources at community events. HHCS has identified alternative one-time funding to continue the program through the effective date of May 1, 2026. All CFHL services will be discontinued as of May 1, 2026, and staffing will be redirected to other program areas with alternative available funding for the remainder of FY26.

4. Services to Enhance Early Development (SEED) On February 2, 2026, Berkeley Public Health Division received an unofficial communication of an impending cut to the Services to Enhance Early Development (SEED) program funding of $224,393, effective July 1, 2026. This is a revenue-generating contract with the Department of Children and Family Services (DCFS) of the Alameda County Social Services Agency to provide a 0.8 FTE Public Health Nurse and 0.10 FTE Supervising Public Health Nurse. Staff provide services to children and their caregiver(s) throughout the county including Berkeley. They also coordinate and

provide information to their counterparts (Child Welfare Workers, mental health clinicians, and parent advocates) in the SEED team regarding the ongoing medical needs of the child. The program's core activities in Berkeley include entering and reviewing information such as birth records, medical/dental records, immunization records, laboratory, and radiological reports in the Child Welfare System (CWS)/Case Management (CMS) and Health and Education Passport (HEP). The target population is Alameda County juvenile court-dependent children between the ages of zero to three years and 11 months. HHCS is currently working to shift members to other vacant grant-funded activities. The program currently only provides services to one child in Berkeley and will have minimal impact on services.

5. Impact of Measure GG Revenue Transfer to Fire Department: The Department's emergency preparedness capacity will be significantly diminished due to the mandatory transfer of $339,487 in Measure GG funding to the Fire Department. To accommodate this loss of revenue, HHCS will be forced to eliminate 2 existing Full-Time Equivalent (FTE) positions within our preparedness programs. It is important to note that while these workforce reductions are necessary to balance the departmental budget and are included in our overall recommendations, they generate zero net savings for the General Fund or the Department. Instead, this action simply shifts the funding source to another department, resulting in a direct loss of HHCS staff and a reduction in our ability to coordinate comprehensive city-wide emergency readiness and response initiatives.

Our Strategic Approach

In navigating these difficult choices, HHCS rejected a "one-size-fits-all" methodology. We did not consistently utilize across-the-board cuts to contracts because many programs rely on the General Fund as a critical match for external grants, and others are not General Fund-supported at all. Instead, we employed a surgical approach rather than an "ax." This precision allows us to protect leveraged resources, honor our commitment to equity, and prevent the inadvertent loss of grant revenue.

Guiding Principles

The following principles served as the "north star" for all departmental decision-making:

- **Financial Health:** Prioritizing permanent structural adjustments over one-time fixes to ensure long-term stability.
- **Equity and Access:** Maintaining a commitment to meeting the needs of Black and Brown residents in underserved communities.
- **Impact Mitigation:** Prioritizing reductions that result in the "least impact" to direct community services.
- **Core Service Protection:** Ensuring reductions do not compromise our mandate to provide a resilient, safe, and prepared City.

- **Consistency with Public Intent:** Honoring the original intent of voter-approved taxes (e.g., the Sugar-Sweetened Beverage tax). This will be reduced to zero recommended reductions in the next iteration.

Strategies Employed:

- **Vacancy Management and Reduction in Workforce:** Due to projected deficits in specialty revenue, the department had been holding positions vacant to avoid reductions to filled positions. As a result, the department has a large number of vacancies recommended for elimination.
- **Alternative Funding Swaps:** Shifting General Fund (GF) expenditures to Childcare Mitigation funds, or existing grant or other funding stream balances (BHSA, Realignment, WIC, U1, PLHA, etc.).
- **Revenue Optimization:** Increasing Environmental Health permit fees to align with actual costs and neighboring jurisdictions.
- **Service Realignment:** Reducing or eliminating local funding for services that are legally the responsibility of Federal, State (Regional Center/Medi-Cal), or County systems.
- **Contract Reductions:** A variety of logic frameworks were utilized and detailed in the budget workbook to propose reductions depending on the service, impact, and funding stream.

Division Specific Comments

1. Office of the Director

- **Youth Equity Partnership:** Shifted select costs to the Childcare fund. To honor equity principles, contract reductions were limited to 2.5% in the initial 10% scenario, increasing to 5% under this 12.5% recommendation.

2. Aging

- **Meals on Wheels $78,542:** Shifted to Fund 111 (Donations) for FY27. This is a one-time shift supported by current fund balances. Additional departmental cuts are recommended for FY28.

3. Environmental Health

- **Admin Support:** Elimination of 1.0 FTE vacant Office Specialist II position. Modernized database software will streamline intake to mitigate the reduction in support staff.
- **Vector Services:** HHCS is recommending the permanent transfer of vector control services to Alameda County. While these services are funded through a separate,

dedicated tax, the City has been unable to meet its contractual obligations to the County as far back as 2021. This failure to meet core requirements prevented the City from fully recouping dedicated funding, necessitating a continuous and unsustainable supplement from the General Fund. Recently, due to expired staff credentials and persistent vacancies, the County began performing vector services directly for the City. This transition resulted in a very positive operational experience and, importantly, a higher level of service for our residents. By formalizing this transfer, we can ensure consistent service delivery while eliminating the need for General Fund subsidies.

4. Mental Health

The Mental Health Division is recommending a reduction of 44 positions. While this appears to be a large number, it primarily represents a cleanup of vacant positions “on the books” and "right-sizing" of the division following an ambitious expansion period. Approximately four years ago, the mental health division scaled up staffing by approximately 10 new positions in anticipation of CalAIM’s promise of expanded Medi-Cal populations and increased service needs. However, mid-expansion, several vital changes occurred to state funding. The State shifted Medi-Cal to a fee-for-service model and implemented BHSA/Prop 1 changes. As a small jurisdiction acting as a contractor to a large county for MediCAL, Berkeley was disproportionately penalized by these rate cuts effectively reducing our Medi-Cal revenue by 50%. In addition, BHSA (Prop 1) diverted treatment funds toward state-level prevention and housing mandates. Recognizing these fiscal shifts early on, the Division had the foresight to hold many of these expansion positions vacant. It’s important to note that over the past four years mental health had an average of 24 vacant positions. Consequently, only 7 of the 44 proposed reductions are currently filled. Because many of these roles were never implemented due to the state’s funding reversals, and adjustments in service due to staffing levels, their elimination will not result in further loss of established community services. When the City is unable to provide required MediCAL covered services individuals are referred into the County system. The only significant impacts to services provided to the community are included in the Mobile Crisis recommendation.

- **Mobile Crisis Team:** Sadly, the only GF funding left in this division funds 2.6 FTEs. While this will be challenged, crisis response is not the responsibility of cities in California. Berkeley’s ability to subsidize the County’s responsibility was a luxury of surplus funding; however, facing a structural deficit, difficult recommendations must be made. The proposed elimination of the Mobile Crisis Team would result in the loss of Berkeley’s only directly operated local mental health crisis response, shifting the primary responsibility for wellness checks and psychiatric emergencies to the Berkeley Police Department. The landscape has also changed significantly since we discontinued the Specialized Care Unit, making reliance on external providers like Alameda County’s CATT questionable. That program is currently facing its own 33% budget reduction and the potential loss of daytime service

teams. Furthermore, Governor Newsom's 2027 budget proposal to make the Medi-Cal Mobile Crisis Service benefit optional (no longer mandated) makes it highly unlikely that county-level alternatives will remain readily available.

5. Public Health

- **Heart to Heart:** The proposed elimination of the Heart-to-Heart program (1.0 FTE filled, 1.0 FTE vacant) is a difficult but necessary recommendation driven by the transition toward a more sustainable fiscal model. While the program has provided valuable community-based blood pressure screenings and health education, these services are increasingly available through federally qualified health centers (FQHCs) and primary care providers who are better equipped to integrate long-term clinical management into patient care. By eliminating this program, the Department can consolidate its limited General Fund resources toward core mandated public health functions that protect the entire community's safety and resilience, rather than maintaining specialized, non-mandated outreach efforts that duplicate services existing elsewhere in the local healthcare ecosystem.
- **Targeted Case Management:** Elimination of 1.0 FTE vacant Public Health Nurse position; tasks absorbed by Social Services Specialist staff.
- **WIC Program:** Shifted 40% FTE from GF to the WIC grant, which has shown the capacity to cover salary costs without impacting service levels.
- **Sugar-Sweetened Beverage (SSB) Contracts:** Minimal percentage reductions to meet targets while honoring the original intent of the voter-approved tax.
- **Through the Looking Glass:** Reductions in OT and mental health services for developmental disabilities. These are mandated responsibilities of the State Regional Center or BUSD (via IEPs) and are eligible for Medi-Cal billing. This service has been valuable for approximately 150 individuals that have difficulty navigating the Regional Center system of services or accessing them through IEPs. That said, they are available if deemed medically necessary.

6. Housing

- **Staffing:** Similar to other divisions, Housing held vacated positions vacant and eliminated vacant positions rather than filled ones. This includes 1 additional GF-funded FTE (Labor PM1), plus several vacant special-funded positions due to increased cost burden on special funds from COLAs. While the reduction is positions is impactful, it is not generally anticipated to reduce overall service delivery. Rather, it will increase the workload for existing staff and will impact responsiveness for certain programs (including Shelter Plus Care and Labor Standards). It will also reduce overall outreach and education for our policies and programs.

- **Community Agencies:** Key strategies included shifting funding to special funds where possible, including utilizing PLHA for the STAIR Center and Measure U1 for Step Up Housing.

- **Rationale for other program-specific reductions:**
    - **Berkeley Winter Shelter:** This recommendation is being made because currently there is no feasible location for the shelter. If adopted, this recommendation will result in the loss of 25 beds during the winter season. These beds were primarily occupied by the most vulnerable unhoused folks living on the street, often seniors who are resistant to moving into congregate shelters.
    - **Homeless Resource Center:** 8% reduction to core operations (excluding direct cash assistance programs).

        - *Alternative:* It's possible the reduction may need to be spread across some of the cash assistance programs pending meeting with BACS but currently proposing it to be applied to core operations only.
        - **Alternative 2:** This reduction could also be offset by the elimination of the Measure P funded temp CSS2 that is currently in HCS's baseline budget. Since being notified by HR that this position can't be extended, there will be an additional $176K each year. This could be used to restore HRC and a portion of Step Up.

    - **STAIR Center:** Berkeley is the SOLE funder for the program, even though participants access the shelter through Coordinated Entry due to Federal funding through COB (ESG). Staff recommends BACS seek to diversify funding for this program. Further, the program capacity decreased by 25% when they shifted to the non-congregate shelter program. Staff still need to meet with BACS to better understand the full impact on the budget. This is a 23% reduction to operating funding.
    - **Step Up Housing:** Currently proposing a 22% reduction with the remaining funds shifting to U1. Berkeley is the SOLE funder for all operations outside of case management services. Units are filled through Coordinated Entry, and while the majority of placements are North County, it is not exclusively north county, nor exclusively Berkeley. Staff recommend decreased reliance on Berkeley funding, increased leverage of local funds, and increased diversification of funding to ensure longer-term sustainability of the program. This represents a 22% reduction to operations.
        - *Alternative:* Restore program funding but shift 100% of program cost to U1, and if necessary, reduce the HTF allocation to accommodate.

- *Alternative 2:* Restore a portion of this reduction with the Temp CSS2 MP funded position currently in HHCS's baseline budget, with the remaining portion being added to U1 with a corresponding reduction to the HTF allocation

Note: The U1 budget Rev/Exp has not been analyzed yet (as far as I'm aware)

- **Lifelong Medical Care, Trust Clinic:** As an FQHC, Lifelong is mandated by the Public Health Service Act to provide primary care regardless of ability to pay. Their enhanced federal reimbursement rates and cost-settled prospective payment systems are designed to cover indigent care, reducing the need for City subsidies for basic clinical services. As part of the surgical approach to fiscal sustainability, HHCS is recommending a 4-year transition for the Trust Clinic contract. Reducing the allocation by approximately 25% each year until the General Fund obligation is eliminated. It should also be noted that the total number of uninsured individuals in Berkeley has dropped significantly from 9,000 before the passage of the Affordable Care Act to 3,400 in 2024 (see chart at end of memo).

  It's also significant to note that prior to the FY25/26 Budget, the Trust Clinic was funded in FY24 for $365,560. It was only Increased in FY25/26 to $525K each year to fund tenant improvements to the Clinic. The reduction in FY27 still results in funding above the FY24 level, or $394,000 although FY28 will represent a 28% reduction from $365,560 for a total of $263,000.

- **Lifelong Medical Care (Seniors):** Low-income older adults aged 60 and over, regardless of ability to pay. In addition to funding for the Trust Clinic, the City provides Lifelong $114,543 annually for services to seniors. Again, the services provided are mandates of FQHCs. The funding provided by Berkeley has assisted with reduced wait times and supported Lifelong's overall fiscal viability. However, facing a structural deficit and the fact that the uninsured population in Berkeley has dropped significantly, HHCS felt it was reasonable to bring this recommendation forward.

7. Other Highlights

- **NEED:** NEED receives most of their supplies from the State and utilizes volunteer services; we believe they will be able to continue services despite this reduction.
- **Berkeley Free Clinic (BFC):** Elimination of the $50k annual laboratory contract. This decade-long partnership has seen a dramatic decline in uninsured clients following the implementation of the ACA (see below).

  *Encounters vs Revenue***

  - 2015: 11,540 total encounters | $738,137

- 2017: 11,102 total encounters | $894,809
- 2019: 9,451 total encounters | $949,275
- 2021: 5,096 total encounters | $651,328
- 2023: 5,459 total encounters | $1,113,875
- 2024: 7,378 total encounters | $1,225,569

Recommendation: Consolidation of Community Health Commission

In alignment with the City Council's 2021 Systems Alignment initiative and the ongoing phased approach to Commission Reorganization, I recommend the consolidation of the Community Health Commission (CHC) into the Health, Life Enrichment, Equity & Community (HLEEC) Committee as a standing subcommittee. Currently, the CHC experiences inconsistent meeting schedules and vacancies that delay policy input. By transitioning the CHC into a Subject Matter Advisory Subcommittee of HLEEC, the City can maintain resident expertise while ensuring that health policy recommendations are directly integrated into the Council's workflow, saving significant staff hours within the HHCS Department.

This proposed change mirrors the Council's previous actions to "right-size" civic engagement by reducing redundant meeting structures that often result in high vacancy rates and inconsistent quorums. By transitioning the Commission's functions to a specialized advisory subcommittee under the HLEEC, the City can preserve vital resident expertise while significantly enhancing staff capacity. In response to significant Public Health funding reductions, this shift will allow Public Health staff to pivot from high-volume administrative meeting support toward the direct implementation of Council health priorities, thereby maximizing the impact of General Fund resources during this stabilization period.

Conclusion

HHCS remains committed to fiscal responsibility while prioritizing the essential needs of our community. We understand that as fiscal conditions evolve and further analysis is completed, our strategy may need to shift. We stand ready to pivot our approach, adjust these recommendations as new revenue data becomes available, or expand upon these concepts to ensure the City's long-term stability. We are available to answer any questions and provide additional details to support City Leadership through these difficult decisions.

Number of uninsured people (Berkeley, CA)

- 2010–2014: 8,978
- 2015–2019: 3,565
- 2020–2024 (most recent 5-year period): 3,396

U.S. Census Bureau, American Community Survey (ACS) 5-Year Estimates, Table B27001 ("Health Insurance Coverage Status by Sex by Age"), accessed via mySidewalk for Berkeley city, California.

